

Patient Name : XXXXXXXXXX
Date of Birth : XXXXXX
Patient Sex : XXXXXX
Study Date : XXXXXXXXXX

Site Name : XXXXXXXXXX
Ref. Physician : XXXXXXXXXX

DIAGNOSTIC IMAGING REPORT

RADIOGRAPHIC EXAMINATION - CERVICAL SPINE

INDICATIONS: Images received for secondary radiographic consultation. Neck pain and headaches. MVC in January 2022. Concern for instability

COMPARISON: None.

FINDINGS:

Five views of the cervical spine to include APLC, APOM, neutral lateral, flexed lateral, and extended lateral views.

Flattening of the cervical lordosis is seen along with a mild anterior head carriage. There is translational motion between the flexed and extended lateral views of multiple vertebral motion segments, primarily involving C3-C4 and C5-C6 measuring approximately 2.48 mm and 3.03 mm, respectively. Please see level by level analysis below. The atlantodental interval and paraodontoid spaces are maintained. Overall osseous density is maintained. There is mild enthesophyte formation seen at the external occipital protuberance.

There is no evidence of acute fracture or dislocation. Vertebral body heights are maintained. Mild C4-C5 and C5-C6 intervertebral disc space narrowing is seen along with spondylotic spurring and hypertrophic uncovertebral degeneration that is slightly more prominent on the right. No evidence of osseous neuroforaminal encroachment.

The prevertebral soft tissues are maintained.

IMPRESSIONS:

1. No acute fracture or dislocation.
2. Translational motion of multilevel vertebral motion segments, however is most prominent at C3-C4 and C5-C6 measuring approximately 2.48 mm and 3.03 mm which may be suggestive of ligamentous laxity/injury. See below for level-by-level analysis.
3. Mild C4-C5 and C5-C6 spondylosis and uncovertebral arthrosis that is slightly more prominent on the right.
4. Mild enthesopathy at the external occipital protuberance.
5. Flattening of the cervical lordosis that may be seen with muscle spasm.
6. Mild anterior head carriage.

Levels	Flexed Lateral	Extended Lateral	Results
Note: Criteria for instability in the cervical spine according to AMA Guides to the Evaluation of Permanent Impairment is translational motion greater than 3.5 mm. Anterolisthesis (positive value) and retrolisthesis (negative value).			
C2-C3	1.04 mm	-0.56 mm	1.6 mm
C3-C4	0.12 mm	-2.38 mm	2.48 mm
C4-C5	-0.05 mm	-1.99 mm	1.94 mm
C5-C6	0.02 mm	-3.01 mm	3.03 mm
C6-C7	0.65 mm	-0.58 mm	0.91 mm

Electronically signed by

Rishi T Bodalia DC MS DACBR RMSK

Thank you for allowing our participation in your patient care.